

PSYCHOSOCIAL DEVELOPMENT

NCLEX STUDY GUIDE

Developmental theories • Milestones • Play • Safety

1. Piaget Cognitive Development

Concept	NCLEX meaning
Schema	Organized mental framework used to understand the world.
Assimilation	Incorporating new ideas and experiences into an existing framework.
Accommodation	Changing the mental framework so a new experience can be added.

Stage	Age	Key findings
Sensorimotor	Birth-2 yr	Coordinates senses with motor responses; sensory curiosity; object permanence develops; language is used for demands and labeling.
Preoperational	2-7 yr	Symbolic thinking, strong imagination and intuition; egocentrism expected; death viewed as reversible; conservation develops after about age 7.
Concrete operational	7-11 yr	Logical thinking tied to concrete situations; understands time, space and quantity.
Formal operational	11+ yr	Theoretical, hypothetical and abstract reasoning; applies concepts across contexts.

BOARD FOCUS A preschooler's egocentrism and belief that death is reversible are expected developmental findings.

2. Freud Psychosexual Development

Structure	Development / role
Id	Present at birth; pleasure principle, instinctive and impulsive.
Ego	Begins around 4-5 months; reality testing, problem solving and coping to reduce anxiety.
Superego	Develops about ages 3-6 through interaction and socialization; conscience, values and self-evaluation.

Stage	Age	Primary focus
Oral	Birth-1 yr	Sucking, swallowing and oral gratification.
Anal	1-3 yr	Toilet training; gratification from elimination or retention.
Phallic	3-6 yr	Genital awareness; Oedipus/Electra concepts in classical theory.
Latency	6-12 yr	Sexual impulses are redirected into socially accepted activity.
Genital	12+ yr	Mature sexual interests emerge with puberty.

3. Erikson Psychosocial Development

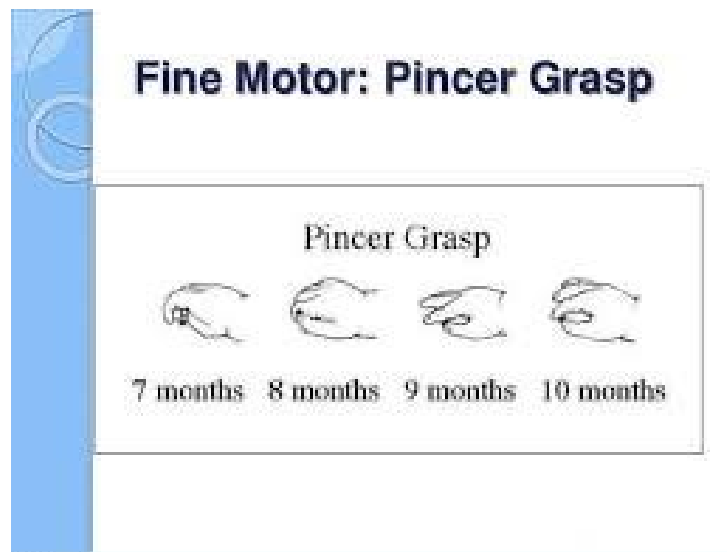
Stage	Age	Clinical cue
Trust vs. mistrust	Birth-18 mo	Reliable, consistent care.
Autonomy vs. shame/doubt	18 mo-3 yr	Independence, "no," tantrums and egocentrism.

Stage	Age	Clinical cue
Initiative vs. guilt	3-5 yr	Exploration, imaginary friends, magical thinking.
Industry vs. inferiority	6-12 yr	School achievement, sports and peer competence.
Identity vs. role confusion	12-19 yr	Peer pressure and development of identity.
Intimacy vs. isolation	20-35 yr	Love, partnership and close relationships.
Generativity vs. stagnation	35-65 yr	Work, society, family and contribution.
Integrity vs. despair	65+ yr	Life review; monitor depression and suicide risk.

4. Infant: Birth to 1 Year



Newborn and infant developmental period



Fine-motor progression: development of the pincer grasp

Growth, nutrition and milestones

Area	Expected finding
Weight	Birth weight doubles by 6 months and triples by 12 months.
Fontanelles	Posterior closes about 3-4 months; anterior closes about 12-18 months.
Dentition	Drooling around 4 months; first tooth 6-9 months; about 6-8 teeth by 1 year. Begin dental care after eruption.
Nutrition	First foods at 4-6 months; iron-fortified rice cereal; introduce one

Area	Expected finding
Motor	food per week; breastfeed at least 6 months; whole milk at 1 year. Head control 3-4 mo; rolls 4-6 mo; supported sitting 6 mo; unsupported sitting 8 mo; crawls and pincer grasp 9-10 mo; stands/walks with help about 12 mo.
Language/social	Social smile about 3 mo; babbling 3-6 mo; “mama/dada” about 8 mo.

SAFETY No honey before age 1. Never put an infant to sleep with a bottle of milk or formula. Use no toothpaste during early infancy.

Play and hospitalization

- Solitary play is expected. Use mobiles/music at 1-2 months, safe rattles at 3-4 months, peekaboo and soft toys at 5-6 months, pots/pans at 9 months, and push-pull toys at 12 months.
- Separation and stranger anxiety develop after about 6 months. Use comfort objects and sucrose on a pacifier during appropriate procedures.

Infant safety

- Use a rear-facing car seat in the middle rear seat according to current height/weight limits; transition only according to the seat manufacturer and current law.
- Avoid balloons and plastic bags; use a one-piece pacifier.
- Keep crib rails secure with slats no more than about 2 $\frac{3}{8}$ inches apart.
- Cover electrical outlets and keep the crib away from windows and curtain cords.

5. Toddler: 1-3 Years



Toddler social development and parallel play

- Walks independently around 13-15 months; goes upstairs two feet per step around 24 months.
- Parallel play: plays beside and imitates another child without sharing.
- Appropriate toys include push-pull toys, large balls and simple four-piece puzzles.

Toilet-training readiness: usually 18-24 months

- Stays dry for about 2 hours and wakes dry from naps.
- Can sit, squat, walk and remove clothing.
- Recognizes the urge to urinate or defecate.
- Wants to please parents and can sit on the toilet for 5-10 minutes.

Behavior, nutrition and safety

- Maintain routines, avoid abrupt endings and offer only two acceptable choices.
- During a tantrum, ignore the behavior while maintaining safety; do not lock the child alone in a room.
- Finger foods are appropriate. Physiologic appetite decreases; excessive milk can contribute to iron-deficiency anemia.
- Keep medicines, poisons and plants locked; secure stairs and bathrooms; turn pot handles inward; keep appliance cords out of reach. Syrup of ipecac is contraindicated.

NCLEX TERM Animism is the belief that toys and other objects have human abilities.

6. Preschooler: 3-5 Years

Age-linked skill	Expected finding
3 years	Rides a tricycle, copies a circle, walks backward and downstairs.
4 years	Hops on one foot; catches and throws a ball.
4-5 years	Uses scissors and undresses without help.
3-5 years	Transient stuttering may occur; do not repeatedly correct the child in front of others.

- Magical thinking and imaginary friends are expected; do not reinforce the fantasy as reality.
- For nightmares, check the room with the child and provide comfort until ready to sleep.
- Associative play: children share materials but do not consistently follow rules. Role play, coloring, sand, playground equipment, tricycles and construction toys are appropriate.
- Regression may occur with hospitalization. Allow play with safe medical equipment such as a stethoscope or needleless syringe.

SAFETY Teach swimming and bicycle-helmet rules, the child's full name/address/parent contact, and how to call 911.

7. School-Age Child: 6-12 Years



School-age growth, learning and cooperative play

- Grows about 2 inches (5 cm) per year; monitor weight percentiles.
- Vision reaches approximately 20/20 by ages 6-7. Greenstick fractures are common.

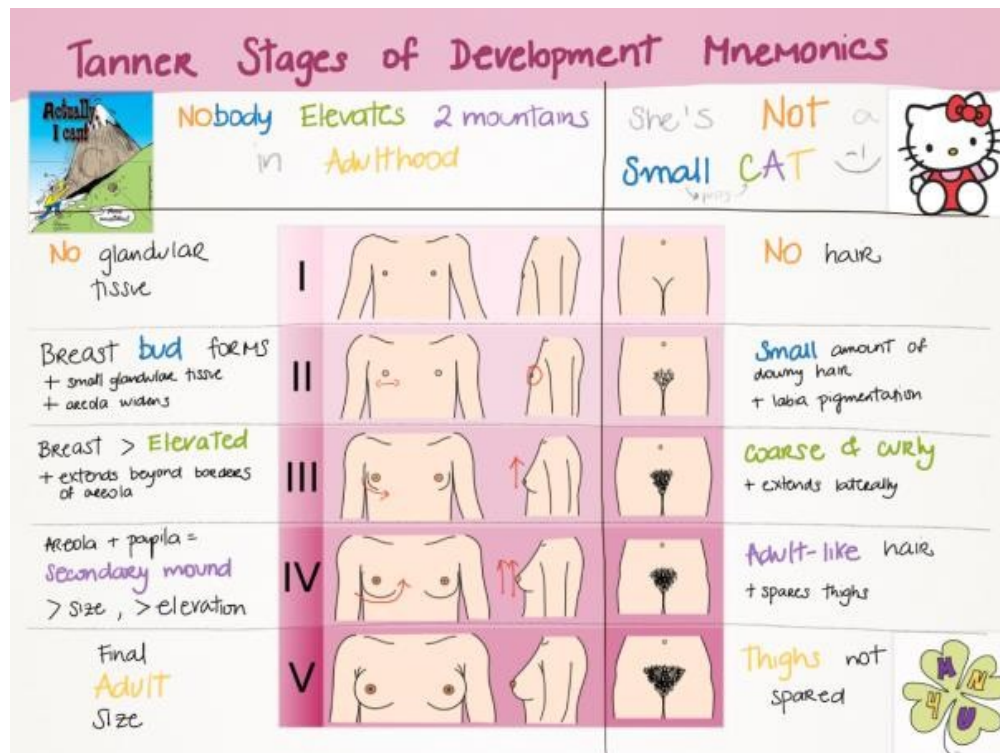
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- Girls usually begin secondary sex changes around ages 9-10; boys around 11-12. Earlier changes require evaluation.
- First deciduous tooth is lost around age 6; by 12 most permanent teeth are present except final molars.
- Cooperative play is expected: swimming, soccer, bicycling, baseball, basketball, construction sets and computer games.

Hospitalization and safety

- Fear of loss of control is common. Explain procedures simply using drawings, diagrams or puppets.
- Assess for child abuse. Introduce age-appropriate sex education, sports safety, and prevention teaching about smoking, alcohol and risk-taking.

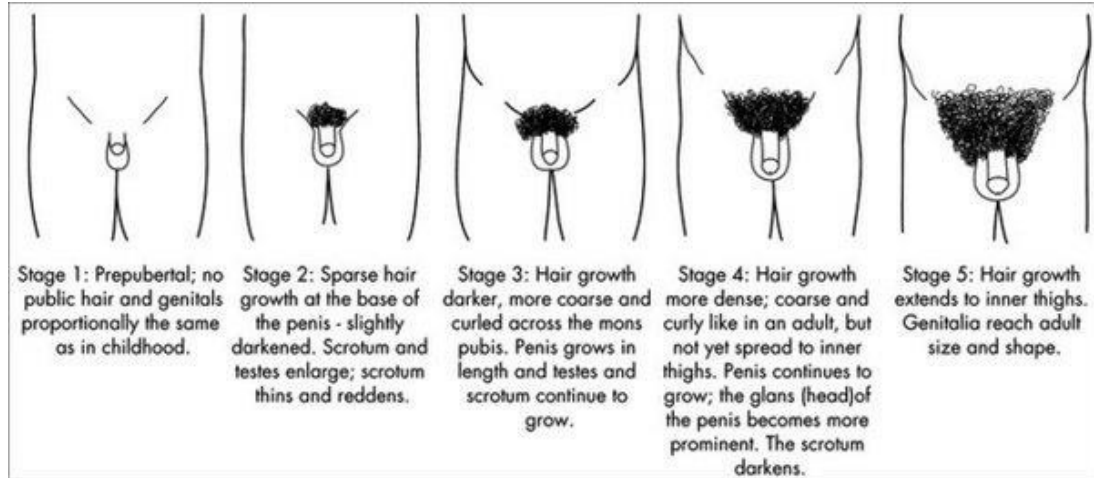
8. Tanner Stages and Adolescence



Tanner stages: visual mnemonic for breast and pubic-hair development

Tanner stages		
Stage	Breast dev	Pubic hair
1	Prepubertal: Papilla elevation only	Prepubertal: No pubic hair
2	Breast bud: Elevation of breast & papilla; enlargement of areola	Sparse, long, slightly pigmented hair on labia majora
3	Further enlargement of breast & areola; no separation of contour	Dark, coarse, curled hair, spreading sparsely over mons
4	Areola & papilla form secondary mound above level of breast	Adult-type hair, abundant, limited to mons
5	Projection of papilla only, recession of areola to contour of breast	Adult-type hair, distribution to the medial thigh
From Arch Dis Child 1969;44:291; J Pediatr 1985;107:317.		

Tanner stages reference table



Male pubic-hair development across Tanner stages

Female breast stage	Description
1	No glandular tissue
2	Breast bud (often ages 9-13)
3	Breast elevation and contour development
4	Secondary mound: areola/papilla project above breast
5	Adult contour

Pubic hair stage	Description
1	None
2	Small amount
3	Coarse, curly and spreading laterally
4	Adult-like hair
5	Extends to thighs

Male stage	Description
1	Prepubertal
2	Testicular/scrotal enlargement
3	Penis length increases; voice changes may begin
4	Penis increases in length and breadth; scrotum darkens; facial hair/ejaculation
5	Adult genital size and hair extending to thighs

Adolescent: 11-19 Years

- Peer pressure and separation may cause anxiety. Involve the adolescent in the plan of care and support peer contact.
- Assess body-image disturbance, low self-esteem, suicide risk, anorexia and bulimia.
- Sebaceous and sweat glands become fully active. Menstruation commonly begins about 2-2½ years after breast development starts.
- Nutrition risks include iron and folate deficiency/anemia.
- Teach injury prevention and risks related to alcohol, drugs, pregnancy, STIs/HIV, piercing, tattooing and excessive tanning.

TERM Echopraxia means involuntary imitation or repetition of another person's movements.

FINAL NCLEX FOCUS Match communication, play, nutrition and safety teaching to the child's developmental stage. Prioritize injury prevention and age-appropriate family support.